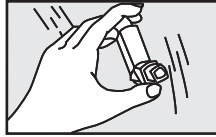


Before using your inhaler for the first time, or if it has not been used for a week or more, "test fire" it, i.e. release one puff into the air.

Using your Inhaler

1. Remove the mouthpiece cover and check that the mouthpiece is clean. Shake the inhaler well. Hold the inhaler upright as shown with your thumb on the base. Place either one or two fingers on top of the canister.



2. Breathe out gently through your mouth.



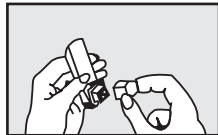
3. Place the mouthpiece of the inhaler in your mouth between your teeth and dose your lips around it (Do not bite it). Start breathing in slowly through your mouth. As you breathe in, press down the canister to release one dose while continuing to breathe in steadily and deeply.



4. Remove the Inhaler from your mouth and hold your breath for 10 seconds, or for as long as it is comfortable. Breathe out slowly.



5. If another dose is required, wait for at least one minute. Repeat steps 2 to 4. After use, replace the mouthpiece cover.



Important

Do not rush steps 3 to 4. It is important that you start breathe in as slowly as possible just before releasing the dose. Practice in front of a mirror for the first



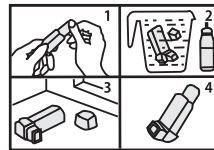
few times, you see "mist" coming from the top of the inhaler or the sides your mouth, start again from step 1. This escaping mist indicates incorrect technique.

6. After inhalation is complete, rinse your mouth with water without swallowing it.

Cleansing

It is essential to keep the plastic mouthpiece clean, to ensure proper functioning of the inhaler. Clean your inhaler at least once a week, as follows:

1. Gently pull the metal canister out of the plastic body of the inhaler. Remove the mouthpiece cover.
2. Immerse the plastic body and the mouthpiece cover in warm water. Do not put the metal canister in water.
3. Next, rinse the plastic body and mouthpiece cover under running tap water.



4. Shake well to remove excess water. Leave to dry. Avoid the use of heat to dry the parts.
5. When the plastic body is dry, replace the canister and the mouthpiece cover correctly.

Warning

1. If the recommended dose does not provide the desired effect, consult your doctor. It is dangerous to exceed the recommended dose.
2. The metal canister is pressurised. Do not puncture or burn it even when apparently empty.
3. Keep away from eyes.
4. Keep away from children.

**For the use of a
Registered Medical
Practitioner or a
Hospital or a
Laboratory only.**

MIDASCARE



MANUFACTURED BY:
**MidasCare
Pharmaceuticals Pvt. Ltd.**
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Cover Page 1

**Salbutamol Pressurised
Inhalation BP
BONAIR™ HFA Inhaler**

NAME AND STRENGTH OF ACTIVE INGREDIENTS

Salbutamol Sulphate BP
equivalent to Salbutamol -- 100
mcg/Actuations.

PRODUCT DESCRIPTION

An aerosol filled in a suitable
pressurised aluminium canister
fitted with a metering valve
provided with oral actuator.

PHARMACODYNAMICS

Bonair HFA inhaler contains a
selective β_2 -adrenergic receptor
agonist, Salbutamol. Salbutamol
acts especially on the bronchi
where it relaxes the smooth
muscles. The effect of salbutamol is
mediated by cyclic AMP.
Salbutamol also inhibits the release
of histamine and some other
mediators in the lungs and
accelerates ciliary clearance of the
bronchi.

PHARMACOKINETICS

Salbutamol is readily absorbed
from the gastro-intestinal tract. It is
subjected to firstpass metabolism in
the liver & possibly in the gut wall,
the main metabolite is an inactive
sulphate conjugate.

It is rapidly excreted in the urine as
metabolized & unchanged drug;

there is some excretion in the
faeces.

Salbutamol does not appear to be
metabolized in the lungs, therefore
its ultimate metabolism & excretion
following inhalation depends upon
the delivery method used, which
determines the proportion of
inhaled Salbutamol relative to the
proportion inadvertently swallowed.

The plasma half-life of Salbutamol
has been estimated as 2 to 7
hours.

INDICATION

Bonair HFA inhaler provides
short-acting (4 to 6 hour)
bronchodilation with fast onset
(within 5 minutes) in reversible
airways obstruction. It is particularly
suitable for the relief & prevention
of asthma symptoms. It should be
used to relieve symptoms when
they occur, & to prevent them in
circumstances recognized by the
patient to precipitate an asthma
attack (e.g. before exercise or
unavoidable allergen exposure).

Bonair HFA inhaler is particularly
valuable as relief medication in
mild, moderate or severe asthma,
provided that reliance on it does not
delay the introduction & use of
regular inhaled corticosteroid
therapy.

RECOMMENDED DOSE

In acute bronchospasm:

Adult dosage: 100 to 200 μ g daily.
Children's: 100 μ g daily. If required,

the dose can be increased to 200
 μ g.

Prevention of exercise-induced bronchospasm:

Adult dosage: 200 μ g daily before
exertion.

Children's: 100 μ g daily before
exertion. If required, the dose can
be increased to 200 μ g.

In Chronic Therapy:

Adult dosage: Up to 200 μ g four
times daily.

Children's: Up to 100 μ g four
times daily.

POSOLOGY & METHOD OF ADMINISTRATION

Administration is by Inhaled route.
Administration of inhalation results
in the rapid onset (i.e. within 5 mins)
of bronchodilation, which lasts for
4-6 hours in most of the Patients.
Increasing use of beta₂ agonist
may be an Indication of worsening
asthma.

For the relief of acute
bronchospasm, 1 to 2 inhalations of
Salbutamol 100 μ g is given. If
Patient with chronic asthma needs
more than one dose of Inhaled beta
agonist daily, then additional
treatment with a corticosteroid has
to be considered. Dosage or
frequency of administration should
only be increased on medical
advice as excessive dosing may
give rise to adverse effect.

CONTRAINDICATIONS

Known hypersensitivity to
Salbutamol sulfate or other

sympathomimetics, or to any other
ingredients of Bonair Inhaler.

SPECIAL WARNING & PRECAUTIONS

The management of asthma should
normally follow a step-wise
programme, and patient's response
should be monitored clinically & by
lung function tests.

Patient should be warned not to
overmedicate as large doses may
produce arrhythmias due to both
the propellants & the active
ingredients.

Hypokalaemia is known to be a
possible side-effect during
treatment with Salbutamol & this
may be enhanced during
concomitant diuretic therapy; in
addition the arrhythmogenic
potential of this interaction may be
clinically important in patient with
ischaemic heart disease.

Cautions: Use In patients with
cardiac disease, hypertension &
diabetes mellitus.

INTERACTIONS WITH OTHER MEDICAMENTS

β -Blockers eg. propranolol, inhibit the
effect of salbutamol. Additional
adrenergic drugs, MAOIs or tricyclic
antidepressants should be used
with caution concomitantly with
salbutamol because of deleterious
cardiovascular effects.

PREGNANCY AND NEONATES

Administration of drug during
pregnancy should only be

considered if the expected benefit
to the mother is greater than any
possible risk to the foetus. Because
no consistent pattern of defects can
be discerned and baseline rate for
congenital anomalies is 2-3% a
relationship with salbutamol use
can not be established.

Unnecessary administration of
drugs during the 1st trimester of
pregnancy is undesirable. As
Salbutamol is probably secreted in
breast milk its use in nursing
mothers is not recommended
unless the expected benefits
outweigh any potential risk. It is not
known whether salbutamol in
breast milk has a harmful effect on
the neonate.

ADVERSE EFFECTS

Includes tachycardia, palpitations,
cardiac arrhythmias, precipitation of
angina pectoris, CNS stimulations,
dry mouth, sweating, headache,
dizziness, nausea, vomiting,
nervousness, anxiety & tremor.

SYMPTOMS AND TREATMENT OF OVERDOSE

Symptoms: Excessive β -stimulation
eg. seizures, angina, hypertension
or hypotension, tachycardia with
rates up to 200bpm, arrhythmias,
nervousness, headache, tremor,
dry mouth, palpitation, nausea,
dizziness, fatigue and insomnia.

Treatment: Consists of
discontinuation of Salbutamol
together with appropriate
symptomatic therapy. Administer

a cardioselective β -adrenergic
blocker (eg., acebutolol, atenolol,
metoprolol), if necessary for cardiac
arrhythmias. However, the
 β -adrenergic blocker should be
used with caution because it could
induce severe bronchospasm.

STORAGE CONDITION

Store below 30°C. Do not freeze.

WARNING:

Pressurised canister. Keep in a
cool place, away from direct
sunlight or heat. Do not puncture or
burn even when apparently empty.

Keep medicine out of reach of
children.

DOSAGE FORM & PACKING AVAILABLE

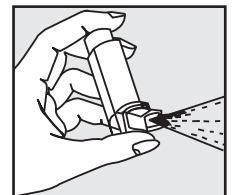
Inhaler 100 mcg / dose x 200 doses

SHELF LIFE

36 Months.

Date of revision: September 2020

HOW TO USE YOUR INHALER CORRECTLY Testing your Inhaler



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